Hackensack Meridian *Health*

Via Electronic Submission - http://www.regulations.gov
The Honorable Thomas Keane
Assistant Secretary for Technology Policy
National Coordinator for Health Information Technology
U.S. Department of Health and Human Services
330 C St SW, Floor 7
Washington, D.C. 20201

February 23, 2026

RE: HHS-ONC-2026-0001-0001, Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care

Dear Assistant Secretary Keane,

On behalf of Hackensack Meridian *Health* (HMH), we thank you for the opportunity to comment on the Department of Health and Human Services (HHS), in collaboration with the Office of the Deputy Secretary and Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology (ASTP/ONC), Request for Information on Accelerating the Adoption and Use of Artificial Intelligence (AI) as part of Clinical Care.[1]

HMH is the largest, most comprehensive, and truly integrated health care network in New Jersey, comprising a network of hospitals that includes three academic medical centers, one university teaching hospital, two children's hospitals, eight community hospitals, a behavioral health hospital, two rehabilitation hospitals and one long-term acute care hospital. Six of our 18 hospitals maintain robust academic medical programs. HMH also has more than 500 patient care locations, including ambulatory care centers, surgery centers, cancer centers, home health services, long-term care and assisted living communities, ambulance services, lifesaving air medical transportation, fitness and wellness centers, rehabilitation centers, urgent care centers, and physician practice locations. HMH has more than 40,000 team members and more than 7,000 physicians within its network and is a distinguished leader in health care philanthropy, committed to the health and well-being of the communities it serves.

As a national leader in clinical innovation, HMH has integrated AI-enabled tools across diagnostics, surgery, care coordination, and administrative workflows to improve outcomes, reduce burden, and enhance patient safety. This includes the use of AI to analyze mammograms in real time to highlight potential signs of breast cancer and improve diagnostic accuracy and timeliness of intervention; AI-assisted robotic precision surgery to enhance surgical accuracy and enable minimally invasive procedures; and AI-driven tools to optimize hospital operations and electronic health record workflows. This strategic and responsible adoption of AI reflects HMH's ongoing commitment to innovation, operational excellence, and high-quality patient-centered care.

Overview of HMH's Comments

[1] 90 Fed. Reg. 60108 (December 23, 2025).

HMH strongly supports HHS's goal of accelerating the responsible adoption of AI in clinical care. AI is no longer experimental in health care and is already embedded across care delivery. Federal policy must therefore evolve from pilot-focused experimentation toward scalable, predictable, and trustworthy deployment. HMH's comments are informed by direct operational experience and focus on how HHS can effectively apply its core policy levers of regulation, reimbursement, and research and development (R&D), to accelerate adoption while protecting patients, clinicians, and health systems.

HMH's comments emphasize that the central challenge facing clinical AI adoption is no longer technological feasibility, but the absence of clear, scalable federal policy frameworks governing accountability, validation, reimbursement, and implementation. Across our responses, HMH underscores that health systems are being asked to assume disproportionate responsibility for evaluating, governing, and managing AI tools without consistent national standards, predictable payment pathways, or clear liability allocation. This uncertainty increases operational risk, slows adoption of high-value tools, and diverts resources away from patient care.

Accordingly, HMH recommends that HHS prioritize a coordinated, risk-based federal approach that enables provider reliance on standardized AI validation and certification, aligns reimbursement with demonstrated clinical and system-level value, clarifies legal responsibility for non-medical device AI, and strengthens interoperability and workforce readiness. As detailed in the specific responses below, federal leadership in these areas would reduce fragmentation, improve patient and clinician trust, and accelerate the safe and responsible integration of AI into routine clinical care.

General Comment Request

HHS seeks stakeholder input on how existing regulatory frameworks, payment policies, and federal R&D investments can be modernized to support the safe, scalable, and value-driven adoption of AI in clinical care.[2]

A. Regulation

HMH supports a predictable, risk-proportionate federal regulatory posture that protects patient safety and privacy without imposing unnecessary or duplicative burdens on providers. As HMH previously emphasized in its comments on the Health Technology Ecosystem Request for Information issued by the Centers for Medicare and Medicaid Services, the primary constraint on AI adoption in clinical care is no longer model performance, but rather operational risk, workflow disruption, governance complexity, and workforce trust.[3]

Existing regulatory frameworks were not designed for AI-enabled care and do not adequately address issues such as model transparency, bias mitigation, explainability, continuous learning, and post-deployment monitoring. In the absence of clear federal standards, health systems are often expected to independently validate and oversee each AI tool they deploy. This approach is operationally unrealistic and diverts limited resources from patient care.

HMH continues to recommend that HHS formally endorse and rely on standardized,

[2] Id. at 60108.
[3] 90 Fed Reg. 21034 (May 16, 2025).

risk-based AI development and governance frameworks such as the National Institute of Standards and Technology (NIST) Artificial Intelligence Risk Management Framework (AI RMF).[4] AI developers should be required to demonstrate compliance through independent audits or certifications. Health systems should be able to rely on such certifications in procurement and deployment decisions, ensuring accountability rests primarily with developers rather than end users.

HMH also reiterates the need to modernize HIPAA and related privacy frameworks to explicitly address AI-specific risks, including re-identification of de-identified data, secondary use of health data for model development, and emerging data modalities such as voice and sensor data. Federal leadership is essential to prevent a fragmented, state-by-state patchwork of AI and privacy requirements that undermines safe and consistent national deployment.

B. Reimbursement

HMH strongly agrees that existing payment policies, particularly encounter-based fee-for-service systems, may unintentionally slow AI adoption or distort incentives.[5] Many of AI's most meaningful benefits, such as reduced clinician burden, improved diagnostic accuracy, workflow optimization, and prevention of downstream adverse events, do not map cleanly to discrete billable encounters.

HMH has previously recommended that Medicare and Medicaid establish clear and predictable reimbursement pathways for AI-enabled technologies and services based on demonstrated clinical impact and value. Reimbursement models should reflect the total cost of ownership associated with AI adoption, including integration, workflow redesign, training, and ongoing support, rather than focusing narrowly on vendor subscription fees or per-use charges.

HHS should approach reimbursement policy for AI-enabled technologies with careful attention to how payment structures shape innovation and adoption decisions. Payment frameworks that emphasize readily quantifiable, encounter-level outputs may unintentionally skew incentives toward tools optimized for billing visibility rather than those delivering meaningful clinical, operational, or preventive value. To avoid these distortions, HHS and CMS should prioritize payment policies that account for AI's full spectrum of value, explicitly recognize contributions to quality, safety, and workforce sustainability, and provide sufficient predictability to support multi-year investment and integration across diverse provider environments.

C. Research and Development

HMH agrees with HHS that federal investments in applied AI research, implementation science, and public-private partnerships can play a critical role in moving promising tools from concept to routine clinical use.[6] ***HMH encourages HHS to focus R&D investments on real-world deployment challenges, including workflow integration, governance models, clinician trust, and post-deployment monitoring.***

[4] https://www.nist.gov/itl/ai-risk-management-framework
[5] Id. at 60109.
[6] Id. at 60110.

HMH believes that conflicting state privacy and AI laws, ambiguous federal guidance on continuous learning AI, and reliance on private consortia to fill federal gaps all impede scalable research and implementation. HHS can address these barriers by providing clear federal direction on data use, model updating, and accountability, and by aligning research initiatives with NIST-led standards rather than fragmented private frameworks.

Specific Question Responses

HHS requests targeted feedback on practical barriers, policy reforms, governance issues, evaluation methods, interoperability needs, and patient concerns shaping AI adoption in clinical care.[7]

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

While interoperability challenges, liability uncertainty, and inconsistent reimbursement remain significant, HMH's operational experience demonstrates that the most consequential barriers to AI adoption in clinical care arise from implementation and governance risk rather than model performance. Currently, health systems must assume responsibility for AI tools embedded in clinical workflows over extended periods, without clear federal standards defining accountability, monitoring expectations, or acceptable risk thresholds. In the absence of standardized federal validation and assurance mechanisms, providers are expected to independently evaluate and oversee each AI tool, which is operationally unrealistic and diverts scarce clinical, compliance, and informatics resources away from patient care.

Workflow disruption and lack of transparency further impede adoption. AI tools that are not seamlessly integrated into electronic health record workflows, generate non-actionable alerts, or lack clear explanation of limitations undermine clinician confidence and sustained use. As HMH has emphasized in prior comments, clinician trust is not built through retrospective accuracy metrics alone, but through consistent real-world performance, explainability at the point of care, and alignment with clinical accountability structures. Absent these elements, even technically sophisticated tools fail to achieve meaningful adoption.

Workforce readiness and regulatory fragmentation compound these challenges. Health systems face a growing need to upskill clinicians, administrators, compliance staff, and governance leaders in AI literacy, risk management, and oversight. At the same time, emerging state-level AI and privacy requirements threaten to create a fragmented compliance environment that undermines national-scale innovation, procurement, and research. Coordinated federal leadership is essential to reduce duplicative oversight, establish clear expectations, and enable responsible deployment across diverse provider settings.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why?

HMH encourages HHS to focus regulatory efforts on predictability, standardization, and appropriate allocation of responsibility. As such, HHS should anchor federal AI policy in the NIST AI RMF and promote standardized, risk-based validation and certification pathways for clinical AI tools.

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[7] Ibid.

Developers should bear responsibility for demonstrating compliance through independent assessment, while health systems should be able to rely on such certifications rather than duplicating validation and assurance efforts for each deployment. This approach would better align accountability with those best positioned to manage design, data, and model risks.

From a payment perspective, HMH reiterates that encounter-based reimbursement structures are poorly suited to capture AI's contribution to efficiency, safety, and clinician well-being. Many AI-enabled benefits - such as reduced documentation burden, improved triage, earlier detection of clinical deterioration, and avoidance of downstream adverse events - accrue longitudinally and across settings rather than within discrete billable encounters. ***HMH recommends HHS explore reimbursement approaches that recognize AI's system-level value, including through value-based payment models, shared savings arrangements, and programmatic incentives tied to demonstrated outcomes rather than volume.***

Programmatically, HMH believes federal support should extend beyond technology development to include workforce training and implementation capacity. Grants, learning collaboratives, and demonstration programs focused on applied deployment, governance models, and clinician education would address real-world adoption barriers more effectively than additional pilot funding alone. Without parallel investment in workforce readiness and operational infrastructure, even validated AI tools will struggle to achieve scale or sustainability.

3. What novel legal and implementation issues exist [regarding non-medical device AI] and what role, if any, should HHS play to help address them?

HMH has consistently emphasized that liability uncertainty represents one of the most significant deterrents to AI adoption, particularly for AI tools that fall outside the FDA's medical device framework but nonetheless influence clinical decision-making and care delivery. Health systems and clinicians face ambiguity regarding responsibility for harm arising from algorithmic error, data drift, or system failure, despite having limited visibility into or control over model design, training data, and ongoing updates.

Consistent with prior HMH comments, ***HMH recommends that HHS advance a federal, risk-based liability framework for AI in clinical care that aligns responsibility with the entities best positioned to understand, prevent, and mitigate system risks.*** Developers and vendors exercise primary control over model design, training data selection, performance specifications, update cycles, and known limitations. Accordingly, they should bear primary responsibility for AI system safety, reliability, and fitness for intended use. Federal policy should require developers to maintain appropriate liability coverage and provide clear indemnification protections for providers when harm results from defects, performance failures, or undisclosed limitations in AI systems used as intended and in accordance with labeling and contractual requirements.

Hospitals and clinicians that deploy certified AI systems in good faith and in accordance with established standards should be protected from disproportionate liability exposure. Clear federal guidance on accountability for non-medical device AI would reduce legal uncertainty, encourage responsible innovation, and prevent the chilling of adoption of tools that have the potential to meaningfully improve patient outcomes and clinician experience.

4. For non-medical devices, what are the most promising AI evaluation methods (pre- and postdeployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes?

HMH believes that the absence of standardized, scalable evaluation methodologies remains a significant barrier to responsible AI adoption in clinical care. Health systems are often required to independently assess model validity, bias, robustness, and ongoing performance, even when tools are developed and marketed by sophisticated vendors. This fragmented approach creates duplication, increases costs, and introduces variability in patient protections across institutions. Without clear federal benchmarks, evaluation efforts risk being inconsistent, overly burdensome, or insufficiently aligned with real-world clinical risk.

HMH recommends that HHS further operationalize and support standardized, risk-based evaluation methods grounded in NIST's AI RMF. Federal policy should clearly distinguish expectations for high-risk clinical decision-support tools versus lower-risk administrative or operational applications, with proportional requirements for pre-deployment testing, post-deployment monitoring, human-in-the-loop safeguards, and transparency. In addition, HHS should invest in applied evaluation infrastructure, such as shared testing environments, reference datasets, and federally-supported demonstration projects, to reduce duplicative burden on providers while improving overall safety and trust.

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

HMH cautions strongly against reliance on private or quasi-regulatory consortia to establish AI assurance standards in the absence of clear federal leadership. As HMH has previously noted, when federal standards are unclear or underdeveloped, private entities such as the Coalition for Health AI have sought to fill the void by issuing their own validation frameworks and assurance mechanisms.[8] While collaboration with industry and academia can be valuable, delegating de facto regulatory authority to private consortia risks concentrating influence among large incumbents, creating inconsistent standards, and erecting barriers for smaller or emerging developers.

HMH therefore supports the Administration's scrutiny of private or quasi-regulatory AI assurance efforts operating outside formal federal oversight and strongly recommends that HHS, in collaboration with ASTP/ONC, anchor AI assurance efforts on NIST-led standards and risk management frameworks as the foundation for responsible AI development in health care. HMH believes NIST is best positioned to lead this work given its technical expertise, transparent approach, and established role in consensus-based standards development. This approach should be reinforced through coordination with the National Committee for Quality Assurance and the National Quality Forum to ensure alignment with quality measurement, accreditation, patient safety, and interoperability objectives. Clear federal endorsement of the NIST AI RMF would provide consistent, nationally recognized criteria for validation, fairness, and accountability, accelerating responsible AI adoption while maintaining

8 https://www.jointcommission.org/en-us/knowledge-library/news/2025-09-jc-and-chai-release-initial-guidance-to-support-responsible-ai-adoption

patient safety and public trust. Importantly, certified status should be meaningful and actionable, allowing health systems to reasonably rely on certification without duplicating evaluations internally.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care?

Interoperability remains a foundational enabler of safe and scalable AI adoption. While progress has been made through Fast Healthcare Interoperability Resources (FHIR)-based standards and application programming interface (API) access, significant gaps persist across clinical, claims, imaging, and patient-generated data sources. Fragmented data access limits the ability of AI tools to deliver accurate, equitable, and context-aware insights, and increases integration costs for providers.

HMH recommends that HHS continue to prioritize and expand FHIR-native interoperability requirements, standardized APIs, and data access policies that support real-time and longitudinal data use. Federal efforts to promote consistent data quality standards and benchmarking would further accelerate AI development and deployment while reducing integration burden on health systems. Improved interoperability would not only support innovation, but also enhance competition, reduce vendor lock-in, and improve patient outcomes.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care?

HMH believes patient trust is essential to the responsible use of AI in clinical care. AI-enabled tools can improve access, navigation, safety, and personalization of care, but these benefits will only be realized if patients understand how AI is used and have confidence in the safeguards protecting their data and rights.

HMH recommends that federal policy prioritize transparency, strong privacy protections, and patient-centered communication regarding AI-enabled tools. Patients should have clear information about when and how AI is used, how their data are protected, and what safeguards are in place to address bias and error. HHS should also encourage mechanisms for patient feedback and choice where appropriate. Centering patient trust in AI policy will be essential to sustaining adoption and delivering on AI's promise in health care.

Conclusion

HMH appreciates HHS's leadership in advancing responsible AI adoption in clinical care. HMH welcomes continued engagement with HHS and the opportunity for in-person discussion on how targeted reforms to regulation, reimbursement, and R&D can help accelerate responsible AI adoption that enhances patient care, preserves trust, and sustains innovation.

If you have questions, please feel free to contact me at 201-370-8279 or Sarah.Lechner@hmhn.org.

Sincerely,

Sarah Lechner
SVP, Chief of External Affairs